

جوستاف روسي انترناسيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : سهير مورييس جرجس قسطندي
المحافظة : الجيزة
الحي / المدينة : ابوالنمرس

العنوان : --
تاريخ تسجيل الإستمارة : 20-09-2026

رقم البطاقة : 27706292101149 رقم التليفون : 01286421722
العمر(تاريخ الميلاد) : 29-06-1977 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطه العلاج : علاج مبادة سرطان الثدي العلاج المساعد لوقاية العظام للمرضى
الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157]
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
 الاسم: د/ندى حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناقشة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 نفقات :
 ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: سهير مورييس جرجيس قسطندي
National ID: 27706292101149



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وماعة الخروج :
PM 01:17 05-07-2026

تاريخ وماعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



١٥ م
هـ سبت
تعاليل دفعو هـ



Patient's Name: Soheir Morris Girgis
Date: 2026-05-21
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

I- CHEST:

- Known case of right breast cancer underwent right mastectomy
- Right lower lobar small nodule of ground glass opacity and spiculated margin measuring 12 x 11 mm
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Internal fixation of L3, L4 and L5 with mild anterolisthesis of L4

Opinion:

- ❖ Known case of right breast cancer and right mastectomy.
- ❖ Right lower lobar nodule ?? metastatic

Dr. Asser Ahmed



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name: **فايزة موزين جرويس قسطنطين**

Sex:

3- Medical number:

Female

83267706

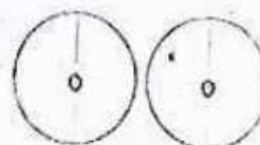
Old Med:

Date:

9/3-2025

2-ID number: 27706292101149

Patient Age: 48 Y



Family history

Family history

Phone

Phone

01503154330

BMI

BMI

REASON OF MDT

REASON OF MDT

Marital status

Marital status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co-morbidity

Co-morbidity

+ve

Menopausal status

Menopausal status

Initial STAGE

Initial STAGE

2025-05-25

Clinical data

Breast Imaging :

Mets Workup :

pet ct 23/1/2025:metabolically active solitary retro-pectoral nodal lesion 0.7x1.2 SUV 9.3

BX IDC	ER 3/8	PR 6/8	Her 2 Neu 2	SISH	KI67 <20%
BX COMMENT	ER COMMENT	PR COMMENT	HER2 COMMENT	SISH COMMENT	KI67 COMMENT

T	N	M
		1

Sonomammography

PATIENT HISTORY

2016 RT MRM -->adj chemo 3 FEC+9 wks taxol-->PORT+tamoxfin x 2 years then ovarian ablation by RTH+femara x 5 years ended 6/2023 kept under follow up 1/2024
infracalviclar LN FNAC-->+ve malignant received aromasine 1/2024 till now
pet ct:metabolically active solitary retro-pectoral nodal lesion 0.7x1.2 SUV 9.3
infracalviclar LN trucut biopsy;IDC GII her2 +3

MDT OUTCOME & PLAN

MDT Outcome & Plan

baseline CTCAP

taxol,herceptin IV

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name : الأستاذ / مهنر مورييس جرجس
Patient No. : 27706292101149
Path No. : 1340/25
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13007431
Request Date : 16/04/2025
Verification Date : 08/05/2025
Financial Category : مبادره - مبادره
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Status post right MRM showing infra-clavicular lymph node.

Nature of specimen: Tru cut biopsy, collected at 15/4/2025.

Gross examination

- Three grayish white tissue cores up to 1.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for GATA3, ER, PR, HER2, GATA3 clone (L50-823), ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- * Histological type: Invasive duct carcinoma. (No nodal tissue)
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade II
- * DCIS: No
- * Angiolinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No
- * IPT (Positive control showed appropriate reactivity) :
- * GATA3: Positive
- * ER: Moderate nuclear reaction in 90 % tumor cells (Allred score 7/8)
- * PR: Strong nuclear reaction in 30 % tumor cells (Allred score 6/8)
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)
- * Ki 67: 25 % nuclear reaction of tumor cells

Diagnosis

Infraclavicular lymph node, Tru cut biopsy:
invasive duct carcinoma, Grade II

Verified by :
Dr. Heba Hasan

Printing Date : 11/05/2025 09:28:52

Page 1 of 2

Printing By : marwa mohamed youssef

CAP
ACCREDITED

غرفة رقم (4) - الدور الاول - مبنى رقم (987) - محور نيل النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt

Patient Name : الأستاذة / م. هيل مورييس جرجيس
Patient No. : 27706292101149
Path No. : 1340/25
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13007431
Request Date : 16/04/2025
Verification Date : 08/05/2025
Financial Category : أورام دار السلام - مياكرو
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

IPT:

- * ER: Positive (score 7/8).
- * PR: Positive (score 6/8).
- * HER2: Positive (score 3+).
- * Ki67: Positive in 25% of tumor cells.

Signature
Prof. Dr. Lubna Omar
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

Printing Date : 11/05/2025 09:28:52

Page 2 of 2

Printing By : marwa mohamed youss

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Patient Name: Sohair Morris Gerguis Kostandy	Date : 08/12/2025
Exam : CT CAP+C (CT Chest+Abd. Pelvis)	Age : 48 Year(s)
Medical ID: 98943	Pat. ID: 98943

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

Compared to last study dated 12/8/2025

I Chest:

- Right mastectomy with clear operative bed , no masses
- Small right axillary lymph node 5mm, No sizable axillary L. Ns.
- No sizable hilar or mediastinal L. Ns .
- No sizable pulmonary nodules .
- Otherwise, no consolidations, masses or cavitations.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is average in size with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Clips of cholecystectomy.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys (apart from left renal cortical cyst measuring 6 x5 mm) with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Under-filled urinary bladder hindering its proper assessment.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Scanned skeleton show:

- L3,L4& L5 spinolaminectomy with transpedicular fixation.
- No worrisome lytic or sclerotic lesions in the scanned skeleton.

Opinion:

- Overall stationary course

DR/Reham Khedr ,,,MD



Patient Name: Sohair Morris Gerguis Kostandy	Date : 14/04/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 47 Year(s)
Medical ID: 973/2025	Pat. ID: 98943

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM in

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- The left breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed:
- **Right mastectomy :**
- Right level II interpectoral hypoechoic solid mass measured 1x0.9cm likely infiltrated LN .
- Free right supra clavicular regions.
- **Left breast:**
- No breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Single enlarged left axillary LN at level I showing lobulated cortical thickening reaching 4mm

Opinion:

- Right level II interpectoral hypoechoic solid mass likely infiltrated LN for FNAC.
- Left axillary level I indeterminate LKN for FNAC
- BIRADS4.

Signature Dr. basma mohmaed

14/04/2025 1:12 PM

Name: *Sohair Morris Gerguis*
Referred by: *Prof. Nabil Nasif Rezk*

ID: 4235942
Date: 15/01/2025

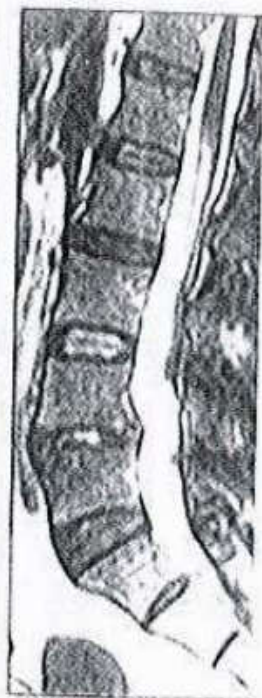
OPEN (1.0 TESLA)
MRI OF THE LUMBOSACRAL SPINE

MRI TECHNIQUE:

- Sagittal: T1 & T2 WI 4 mm sections (plates 1, 2).
- Axial : T1 & T2 WI 4 mm sections (plates 3, 4).
- MR Myelography (plate 5).

MRI FINDINGS:

- Forward slippage of L4 over L5 secondary to bilateral L4 pars interarticularis breaks.
- Reduced bright T2 signal of L2/3 disc and reduced height of L4/5 disc denoting degeneration.
- L5 upper vertebral end plates Schmorl's node with patchy marrow edema of L4/5 opposing end plates likely degenerative.
- L4/5 posterior disc protrusion inclined to left side with mild cranial inclination is seen effacing the anterior epidural fat, indenting the theca and encroaching upon the related neural exit foramina.
- L2/3 minimal posterior disc bulge is seen effacing the anterior epidural space and gently abutting the theca.
- Normal size and signal intensity of the conus medullaris.
- L4 osseous hemangioma.
- No marrow infiltration of the vertebral bodies and their posterior elements.
- No paraspinal soft tissue masses.



OPINION:

- First degree lytic spondylolisthesis of L4.
- L4/5 posterior disc protrusion inclined to left side.
- L2/3 minimal posterior disc bulge.

Dr. Ahmed Sayed, MD

Dr. Nour Elhoda Yehia

Visit Number: 31626000816

Patient Name: سهاير موزيس جرجيس

Age / Sex: 48 year / Female

Patient ID: 2136798343

Client Name:

Referred by Prof:

Exam Date:

Printed:

Referral: عمرو الكتاني

23 Jan 2025

1/23/2025 8:22:14 PM

OT

Bone Mineral Density estimation (Densitometry)**TECHNIQUE:**

- Estimation of the bone mineral density (B.M.D) was achieved at the left hip & left forearm radius as well as the lumbar spine from L1 to L4 level, using dual energy absorptiometry (DEXA) technique.

FINDING:

The BMD of the lumbar spine is 0.908 g/cm^2 , which is 77 % of the mean of the patient's age match and 2.3 SD below maximum mean.

The BMD of the left hip is 0.922 g/cm^2 , which is 94 % of the mean of the patient's age match and 0.7 SD below maximum mean.

The BMD of the left forearm is 0.698 g/cm^2 , which is 80 % of the mean of the patient's age match and 2 SD below maximum mean.

OPINION:

The current findings denote:

- Normal study at the left hip
- Severe osteopenia at the lumbar spine and left forearm.

Dr. Hala M.Sayed , MSc



٥٥ شارع عبدالله امام رباح، برج الأطباء، الدور ٢

٥٩ شارع شبرا

٦٧ شارع المبرغلي

محطة التعاون

كابيتال بزنس بارك، مبنى B5، الدور ٢

٢٠ شارع الجزائر

٢٢ شارع عبد الرزاق السنهوري



Visit Number: 61825560375
Patient Name:
Age / Sex: 47 year / Female
Patient ID: 2136798343 Acc: 1312516

Client Name:
Referred By Prof: Amr El Kafatriy
Exam Date: 23 Jan 2025 00:00
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PT

Whole Body PET/CT Examination

History and Clinical Data: -

Patient data: -

Female patient, 47 years old, Weight: 75 Kg, Blood Glucose level: 120 mg/dl.

Radiopharmaceutical: -

F-18-FDG 5.0mCi injected I.V.

Clinical background: -

Known case of right breast carcinoma, managed in 2016 underwent MRM, CTH, RTH and kept on hormonal therapy.

Now she presents with right infra-clavicular nodal lesion, pathologically proven metastatic.

Purpose of the study: -

For metastatic workup.

No available previous PET-CT study.

TECHNIQUE: -

Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.

Data was processed and displayed according to departmental protocol.

Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.

Liver showed average SUV ~2.15 (max. SUV~3.18).

REPORT: -

Review of the PET, CT and the fused images revealed:

Loco regional (breasts and axillae):

a) Right side:

Clear right mastectomy operative bed with no evidence of local tumoral recurrence.

Metabolically active (FDG avid) right retro-pectoral lymph nodal lesion, measures about 0.7 x 1.2 cm in diameters with SUVmax 9.36.

b) Left side:

No FDG avid parenchymal neoplastic mass lesion detected.

Few small metabolically inactive axillary lymph nodes with preserved fatty hilum, nonspecific.

Visit Number: 61825500375
Patient Name:
Age / Sex: 47 year / Female
Patient ID: 2136798343 Acc: 1312516

Client Name:
Referred By Prof: Amir El Katatny
Exam Date: 23 Jan 2025 00:00
Printed: 24/01/2025 2:21:17 PM

PT

II) Skeleton:

- Grade-I degenerative spondylolisthesis together with increased metabolic activity by L4/5 disc as well as their opposing vertebral endplates associated with cortical erosions achieving SUVmax 7.93, *possibly degenerative with superadded early inflammatory process (discitis).*
- Spondylo-degenerative changes.
- No metabolically active (FDG avid) lytic or sclerotic osseous lesions.

III) No other metabolically active (FDG avid) lesions detected all over the surveyed body that would account for distant metastatic deposits.

IV) The rest of the body:

- Brain: No FDG-avid intra-cranial space occupying lesions.
- Lungs: No FDG avid pulmonary nodules / masses.
- Liver: average in size showing normal FDG uptake with no metabolically active FDG avid focal lesions.
- Spleen: average in size showing normal FDG uptake with no metabolically active FDG avid splenic focal lesions.

Side Findings:

- Hypermetabolic thyroid nodule seen at the postero-inferior pole of the right lobe measuring about 8 mm with SUVmax 4.57.
- Cholecystectomy metallic clips.
- Thickened hypermetabolic left supra-renal gland achieving SUVmax 4.31, for follow up
- Mild circumferential wall thickening of the urinary bladder, likely early cystitis.
- Heterogeneous FDG uptake corresponding to gluteal soft tissue thickening and nodularity (SUVmax 3.86), inflammatory.

Visit Number: 61825500375

Patient Name:

Client Name:

Age / Sex: 47 year / Female

Referred By: Prof. Amr El Katalny

Patient ID: 2136796343 App: 1312516

Exam Date: 23 Jan 2025 00:00

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PT

-) The rest of PET / CT study appears normal.
l) Normal physiological tracer bio-distribution is seen over cerebral hemispheres, heart, bowel, bone marrow and urinary tract.

CONCLUSION:

PET/CT study for a patient known to have managed right breast carcinoma showing:

- Clear right mastectomy bed with no evidence of FDG avid local recurrent neoplasia.
- Metabolically active (FDG avid) solitary right retro-pectoral nodal lesion (metastatic).
- No other FDG-avid distant metastatic deposits.

N.B.: Increased metabolic activity by L4/5 disc as well as their opposing vertebral endplates associated with cortical erosions, *possibly degenerative with superadded early inflammatory process (discitis), for further evaluation by MRI study.*

Prof. Ahmed Wafaie MD, FRCR
Dr. Sally Abdelaziz, MSc



SCAN-8 LABS

مركز النيل للأشعة و التحاليل

Name : Soheir Morris Girgis Kostandi

Date : 22-09-2025 12:26

Branch : Dokki

Medical No. : 208689

Ref.Physician Prof : Gostaf Roseh Hospital

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 48-year-old lady, with history of right breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen all over the surveyed skeleton.
- Degenerative changes are seen at the lower lumbar spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Name : Soheir Morris Girgis
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 10-09-2026 06:52
Medical No. : 458461
تعاقب مستشفى : Ins.Company :

whole body (FDG) PET/CT Study:

Clinical history:

- 49-year-old female patient with history of managed right breast cancer.
- PET/CT is requested for status evaluation.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 3.0

PET/CT findings:

Head and neck:

- Bilateral symmetrical tracer distribution is seen within the brain parenchyma with no abnormal metabolic activity or definite SOLs could be detected.
- Normal appearance of soft tissues and glandular structures of head & neck.
- No pathologically enlarged or tracer-avid cervical or supraclavicular LN.s.

Chest:

- Clear right MRM bed is noted with no definite hypermetabolic sizable mass lesions detected.
- No current significant metabolically active lung nodules or parenchymal infiltrates.
- No enlarged or tracer-avid mediastinal, hilar or axillary LN.s.
- No pleural thickening, masses or effusion.

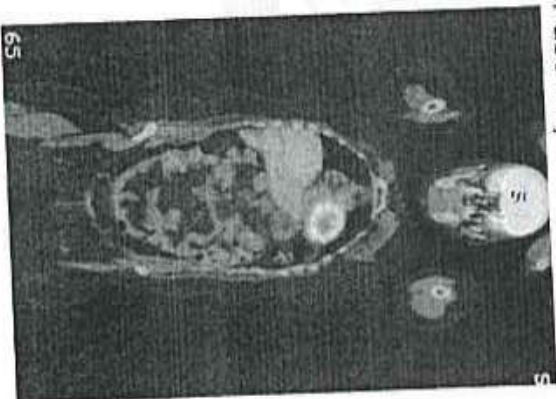
Abdomen and Pelvis:

- Average-sized liver showing homogenous CT-texture and tracer uptake pattern with no definite focal lesions (on non-contrast CT basis.
- Unremarkable spleen, kidneys, adrenals, pancreas and pelvic organs.

Name : Soheir Morris Girgis
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 10-09-2026 06:52
Medical No. : 458461
تعليق مستشفى : Ins.Company :

- No definite gastric or bowel thickening, mural masses (on non-oral contrast CT basis) or tracer-avid foci could be detected.
- No pathologically enlarged or tracer-avid abdomino-pelvic LN.s.
- No evidence of ascites, free or encysted abdomino-pelvic fluid collections.



Musculoskeletal & BM:

- No any pathological sclerotic/lytic bony lesions or abnormal tracer-avid foci could be detected over the surveyed skeleton.

Impression:

Whole-body (FDG) PET/CT study is showing:

- The surveyed body show no scintigraphic evidence of any current significant metabolically active lesions including the right MRM bed.

Best regards,

Dr. Ahmed dowydar, MSc.

Dr. Alaa Nagi, MD.
Nuclear Medicine